

Hand Hygiene Policy

WHO Five Moments, Technique, Compliance
Monitoring & Special Situations

Regulation: 42 CFR §482.42 | CDC/HICPAC |
WHO Guidelines on Hand Hygiene

PURPOSE & SCOPE

Hand hygiene is the single most important measure to prevent the transmission of healthcare-associated infections (HAIs). This policy establishes the requirements for when, how, and with what product hand hygiene must be performed. It applies to all staff, students, volunteers, vendors, and visitors in patient care areas. Non-compliance is a serious patient safety violation subject to corrective action.

THE WHO FIVE MOMENTS FOR HAND HYGIENE

All clinical staff must perform hand hygiene at the following five moments, without exception:

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| 1 | BEFORE touching a patient
Any time you approach and will touch a patient — including to take a pulse, adjust a pillow, assist with positioning, or any other direct patient contact. Protects the patient from pathogens on your hands. |
| 2 | BEFORE a clean or aseptic procedure
Before any invasive procedure, sterile technique, or contact with a non-intact skin site. Examples: inserting IV/urinary catheter, wound care, oral care, medication preparation at bedside, handling sterile equipment. Perform hand hygiene immediately before donning sterile gloves. |
| 3 | AFTER body fluid exposure risk
After contact with blood, body fluids, secretions, excretions, mucous membranes, non-intact skin, wound dressings, or any object that may be contaminated. Perform even if gloves were worn. Protects you and the environment from patient pathogens. |
| 4 | AFTER touching a patient
After any direct patient contact — even if the contact seemed clean. After helping the patient drink, after a handshake, after adjusting a patient's call light. Protects the environment and other patients. |
| 5 | AFTER touching patient surroundings
After touching any object or furniture in the immediate patient environment — even if the patient was not touched. Examples: bed rails, call light, bedside table, IV pump, vital signs equipment, door handle inside patient room. Protects you and others from environmental pathogens. |

PRODUCT SELECTION: ABHR vs. SOAP AND WATER

Alcohol-Based Hand Rub (ABHR) is the preferred method for hand hygiene in most clinical situations due to superior antimicrobial efficacy, faster action, less skin damage, and greater convenience. Soap and water is required in specific clinical situations.

Product/Method	When to Use
ABHR Preferred (all other situations)	Routine patient care in the absence of visible soiling Before and after patient contact (Moments 1, 4, 5) After removing gloves Before and after eating (if hands not visibly soiled)
Soap & Water REQUIRED	Hands visibly soiled with blood, body fluids, or dirt After using the restroom Known or suspected <i>Clostridioides difficile</i> (C. diff) exposure — spores not killed by ABHR Known or suspected norovirus or other non-enveloped virus outbreaks After contact with a patient with known <i>Bacillus anthracis</i> infection
Soap & Water Preferred	Before eating and handling food After potential exposure to spore-forming organisms in outbreak settings

TECHNIQUE — ALCOHOL-BASED HAND RUB (ABHR)

Total time: approximately 20–30 seconds.

1	Apply Product Dispense sufficient ABHR to cover all hand surfaces (approximately 1–3 mL or 2–3 pump actuations). If hands feel dry after rubbing, you did not use enough product.
2	Palm to Palm Rub palms together vigorously in a circular motion until product is distributed across both palms.
3	Palm Over Dorsum Right palm over left dorsum with interlaced fingers; rub. Repeat with left palm over right dorsum.
4	Palm to Palm with Interlaced Fingers Rub palm to palm with fingers interlaced to clean between and around fingers.
5	Backs of Fingers (Rotational) Rub backs of fingers against opposing palms with fingers interlocked (cupped hands).
6	Rotational Rubbing of Thumbs Rotational rubbing of left thumb clasped in right palm and vice versa. Thumbs are frequently missed.

7	Rotational Rubbing of Fingertips Rotational rubbing of clasped fingers of right hand in left palm and vice versa. Clean under fingernails.
8	Allow to Dry Completely Rub hands until product has completely evaporated and hands are dry. Do NOT wipe hands on clothing or paper towel to speed drying — this reduces efficacy.

TECHNIQUE — SOAP AND WATER HANDWASHING

Total time: approximately 40–60 seconds from wet to dry.

1	Wet Hands Wet hands with clean running water (warm preferred, not hot). Avoid splashing. Turn faucet on with paper towel if foot/elbow controls unavailable.
2	Apply Soap Apply sufficient soap to cover all hand surfaces. Antibacterial soap is not required and not preferred over plain soap for routine hand hygiene.
3	Lather and Scrub — 20 Seconds Minimum Lather hands vigorously and scrub all surfaces using the same seven-step WHO technique as described for ABHR above (palm to palm, dorsa, interlaced, backs of fingers, thumbs, fingertips).
4	Rinse Thoroughly Rinse hands under running water. Ensure all soap is removed. Hold hands lower than wrists and elbows to prevent contamination of clean hands.
5	Dry with Single-Use Towel Dry hands thoroughly with a clean, single-use paper towel using a patting motion. Do not rub vigorously as this damages skin and may increase bacterial shedding.
6	Turn Off Faucet Use a fresh dry paper towel to turn off the faucet. Do not re-contaminate clean hands by touching the faucet handle directly.
7	Discard Towel Without Touching Waste Bin Use the paper towel to open the waste bin lid if needed, then discard. Use towel to open the door if room door has a handle rather than a push plate.

GLOVE USE AND HAND HYGIENE INTERACTION

Critical Rule: Gloves Do NOT Replace Hand Hygiene

Gloves must be worn when contact with blood, body fluids, mucous membranes, non-intact skin, or potentially contaminated items is anticipated.

Hand hygiene **MUST** be performed immediately **BEFORE** donning gloves and immediately **AFTER** removing gloves.

Gloves must be changed between patients and between tasks on the same patient when moving from a contaminated body site to a clean body site.

NEVER wear the same pair of gloves for care of more than one patient.

NEVER wash or reuse disposable gloves. Washing gloves does not restore their barrier integrity.

If gloves are torn or punctured during a procedure, remove immediately, perform hand hygiene, and apply new gloves before continuing.

SPECIAL CIRCUMSTANCES & REQUIREMENTS

Surgical Hand Scrub / Surgical Hand Rub

Before any operative or invasive procedure requiring sterile technique, a surgical hand scrub or WHO-approved surgical hand rub must be performed. Surgical hand rub with ABHR is equivalent to aqueous scrub and is preferred. Duration: minimum 3 minutes for surgical hand rub per manufacturer instructions. Traditional scrub with antiseptic (chlorhexidine gluconate 4% or povidone-iodine) requires 3–5 minutes. Wrists and forearms are included in surgical hand hygiene.

Artificial Nails, Nail Polish, and Jewelry

Artificial nails (acrylics, gels, wraps, tips, nail jewelry) are **PROHIBITED** for all staff who provide direct patient care. Natural nails must be kept short ($\leq \frac{1}{4}$ inch beyond fingertip). Nail polish is permitted if not chipped; chipped nail polish harbors organisms and must be removed. Rings with stones or ridges may harbor organisms; a single smooth band ring may be worn but must be removed before surgical procedures. Wrist jewelry must be removed before performing hand hygiene that includes the wrist.

Skin Care and Hand Hygiene Products

Skin integrity is essential for effective hand hygiene. Staff must be provided with and are encouraged to use hospital-approved hand lotion regularly. Personal lotions and creams are prohibited as they may degrade glove material. ABHR dispensers must be mounted at the point of care (outside and inside patient rooms) and stocked at all times. Soap and water facilities must be available in all patient care areas.

COMPLIANCE MONITORING & ACCOUNTABILITY

Monitoring Component	Description
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Direct Observation Audits	Trained observers monitor compliance in clinical areas. Minimum 10 hand hygiene opportunities observed per unit per month. Results are reported to unit managers and posted.
Electronic Monitoring Systems	Where installed, electronic hand hygiene monitoring systems provide real-time compliance data. Data supplements direct observation and is used in performance improvement.
Unit-Level Reporting	Monthly compliance rates are reported to unit managers and shared with unit staff. Trends are tracked. Units below 85% compliance threshold must implement a performance improvement plan.
Leadership Accountability	Unit managers are accountable for staff compliance. Managers must perform hand hygiene observations and coach non-compliant staff in real time. Manager observation rates are tracked.
Corrective Action	Documented non-compliance (observed refusal, repeated non-compliance) is subject to progressive corrective action per HR policy. All staff have a professional and ethical obligation to perform hand hygiene.
Patient and Visitor Empowerment	Patients and visitors must be encouraged to remind staff to perform hand hygiene. Staff must respond positively to patient reminders without defensiveness.

For internal clinical use only. Review annually and upon regulatory updates. This policy does not replace clinical judgment.

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